

AKAMON TABLET 3MG

PRESENTATION

Each white, round, flat, scored tablets embossed MC with diameter of 8mm contains bromazepam 3mg.

THERAPEUTIC INDICATIONS

Bromazepam is a pyridylbenzodiazepine compound with anxiolytic properties.

It is indicated for short term therapy, usually not exceeding two to four weeks, as symptomatic treatment of severe, disabling or causing unacceptable stress, anxiety. The anxiety can be occurring alone or in combination with insomnia or short term psychotic, organic or psychosomatic illness.

PHARMACOLOGY

Pharmacodynamics

Bromazepam is a psychotropic substance of the pyridylbenzodiazepine class and has anxiolytic properties. It binds to specific receptors in the central nervous system and particular peripheral organs. The central nervous system benzodiazepine receptors have a close functional connection with the receptors of the GABA-ergic transmitter system. Following binding to the benzodiazepine receptor the inhibitory effect of the GABA-ergic transmission is augmented by bromazepam.

Pharmacokinetics

Bromazepam is well absorbed from the gastrointestinal tract following oral administration, it is about 84% bioavailable. Peak serum levels are achieved between 1 and 4 hours post administration. The volume of distribution is about 67L. The elimination half-life for bromazepam is between 8 to 20 hours. It undergoes extensive hepatic metabolism with only 2% to 3% unchanged drug being excreted. It is metabolised to the inactive metabolites, 3-hydroxybromazepam and 3-hydroxybenzoylpyridine which are excreted in the urine as glucuronate conjugates and account for 58% to 77% of a dose. Main excretion route is the urine with 52% to 77% of a single dose being excreted in 72 hours. Although there is no specific data in respect of bromazepam, other benzodiazepines cross the placental barrier and are excreted in breast milk.

CONTRAINDICATIONS

Akamon is contraindicated in patients with known sensitivity to bromazepam, other benzodiazepines or any of the tablet ingredients. Akamon is also contraindicated in patients with chronic psychosis, phobic conditions, obsessional conditions, acute pulmonary insufficiency or respiratory depression.

DOSAGE AND ADMINISTRATION

Adult:

The dosage should be individually titrated and optimised, as should frequency of administration, on the basis of the individual response, severity of symptoms and any available history of previous response to psychotropic drugs.

The usual dose in normal circumstances ranges from 3mg to 18mg a day, administered in divided doses.

Hospital patients may, in exceptional conditions, require a daily dosage of up to 60mg in divided doses. The maximum dose of 60mg a day should not be exceeded.

Children:

Not indicated for paediatric use.

Elderly:

Elderly patients are more responsive to bromazepam. The usual dose in normal circumstances ranges from 1.5mg to 9mg a day, administered in divided doses.

Hospitalised elderly patients may, in exceptional conditions, require a daily dosage of up to 30mg in divided doses. The maximum dose of 30mg a day should not be exceeded.

In elderly patients dosage should not exceed those recommended under any circumstances.

Hepatic impairment:

Patients with chronic hepatic impairment will require dose reduction. Care is warranted in such patients with initial dose titration. This is especially important if such patients are also elderly.

Renal impairment:

Patients with chronic renal impairment will require dose reduction. Care is warranted in such patients with initial dose titration. This is especially important if such patients are also elderly.

For all categories of patient, the lowest dose that control symptoms should be used. The use of the full dosage regime should not be prolonged beyond four weeks. Withdrawal of therapy should be done by gradual dose reduction. This may require more time the longer a patient has been on therapy, and in such cases specialist therapy may be appropriate.

AKAMON tablets are for oral administration.

SPECIAL WARNINGS AND PRECAUTIONS FOR USE

Patients with chronic pulmonary insufficiency, hepatic impairment and renal impairment may require dosage reduction.

As the 1.5mg strength tablet contains tartrazine it should not be administered to patients known to be sensitive to tartrazine.

Amnesia may occur.

Akamon should not be used alone to treat anxiety associated with depression or depression as it may precipitate suicide in such patients.

Use of Akamon in cases of loss or bereavement may inhibit psychological adjustment.

There are reports of abnormal psychological responses to benzodiazepine therapy; these rare behavioural effects include paradoxical aggressive outbursts, excitement and confusion. Depression with suicidal tendencies has been uncovered during therapy. It is recommended that extreme caution be exercised in the administration of benzodiazepines to patients with disorders of the personality.

Caution should be observed in administering Akamon to patients with myasthenia gravis due to the pre-existing muscle weakness.

Benzodiazepines have a low potential for dependency, especially when only used short term, although this rises with dosage and duration of therapy. This risk increases in patients with a history of alcohol and/or drug abuse or in patients with marked personality disorders. It is essential such patients are regularly monitored and assessed as to requirement for continuing therapy.

In the administration of Akamon, particularly to the elderly, the very ill patient or patients with limited cardiac or pulmonary reserve, it is necessary to exercise extreme caution due to the possibility of apnea and/or cardiac arrest occurring. Concomitant use of central nervous system depressants, including alcohol and barbiturates, increases the risk of apnea due to increased cardiac or pulmonary depression. Equipment required for resuscitation should be readily available.

Treatment should be withdrawn gradually as sudden therapy cessation may be accompanied by withdrawal symptoms. These are characterised by depression, nervousness, rebound insomnia, irritability, sweating and diarrhoea and have occurred in patients following the sudden cessation of therapy after normal therapeutic doses for a short time.

The sedative effects of Akamon are intensified when used in conjunction with centrally acting drugs and the elderly require special supervision.

Anaphylaxis (severe allergic reaction) and angioedema (severe facial swelling) which can occur as early as the first time the product is taken.

Complex sleep – related behaviours which may include sleep driving, making phone calls, preparing and eating food while asleep.

Risks from Concomitant Use with Opioids

Profound sedation, respiratory depression, coma, and death may result from the concomitant use of Akamon with opioids. Observational studies have demonstrated that concomitant use of opioids and benzodiazepines increases the risks, reserve concomitant prescribing of these drugs for use in patients for whom alternative treatment options are inadequate.

If the decision is made to newly prescribe a benzodiazepines and an opioid together, prescribe the lowest effective dosages and minimum durations of concomitant use.

If the decision is made to prescribe a benzodiazepine in a patient already receiving an opioid, prescribe a lower initial dose of the benzodiazepine than indicated in the absence of an opioid, and titrate based on clinical response.

If the decision is made to prescribe an opioid in a patient already taking a benzodiazepine, prescribe a lower initial dose of the opioid, and titrate based on clinical response.

Follow patients closely for signs and symptoms of respiratory depression and sedation. Advise both patients and caregivers about the risks of respiratory depression and sedation when Akamon is used with opioids. Advise patients not to drive or operate heavy machinery until the effects for concomitant use of the opioid have been determined. Screen patients for risk of substance use disorders, including opioid abuse and misuse, and warn them of risk for overdose and death associated with the use of opioids (See *Drug Interactions*)

INTERACTIONS WITH OTHER MEDICINAL PRODUCTS AND OTHER FORMS OF INTERACTION

Drugs:

Centrally acting drugs: Concomitant use with centrally acting drugs such as anaesthetics, analgesics, antidepressants, hypnotics, neuroleptics and tranquillisers is likely to result in an intensification of sedative effects. The parenteral administration of such drugs in conjunction with parenteral bromazepam may result in severe cardiovascular and respiratory depression occurring. In such instances elderly patients require special supervision.

Analgesics, narcotic: Concurrent administration of intravenous bromazepam with a narcotic analgesic in dental procedures requires careful dose titration of the bromazepam for the patient's needs. In such cases it is recommended that bromazepam be administered after the analgesic.

Anti-epileptic drugs: Results of published pharmacokinetic studies on interaction between diazepam and anti-epileptic drugs have given conflicting results. These include elevation, depression and no change in drug levels. It is likely that the side effects and toxicity of anti-epileptic drugs, particularly the hydantoins or barbiturates may be more noticeable. Extra care should be taken in dose adjustment when initiating treatment.

Inhibitors of hepatic enzymes: Drugs that are known inhibitors of hepatic enzymes, including cimetidine and omeprazole and oral contraceptives, reduce the clearance of benzodiazepines and may potentiate their action.

Inducers of hepatic enzymes: Similarly, known inducers of hepatic enzymes, such as rifampicin, may increase benzodiazepine clearance.

Opioids: Due to additive pharmacologic effect, the concomitant use of opioids with benzodiazepines increases the risk of respiratory depression, profound sedation, coma and death.

The concomitant use of opioids and benzodiazepines increases the risk of respiratory depression because of action at different receptor sites in the central nervous system that control respiration. Opioids interact primarily at μ -receptors, and benzodiazepines interact at GABA_A sites. When opioids and benzodiazepines are combined, the potential for benzodiazepines to significantly worsen opioid-related respiratory depression exists.

Reserve concomitant prescribing of these drugs for use in patients for whom alternative treatment options are inadequate (see *Warnings & Precautions*).

Limit dosage and duration of concomitant use of benzodiazepines and opioids, and follow patients closely for respiratory depression and sedation.

Food:

Alcohol: This may intensify any sedation effect and patient should be cautioned to avoid alcohol during therapy.

FERTILITY, PREGNANCY AND LACTATION

Animal experimentation has not shown the drug to be free from hazard in pregnancy and there is inadequate evidence of safety in human pregnancy. Use in pregnancy is not recommended, especially in the first and third trimester, unless there is overwhelming need and the anticipated benefits outweigh the unknown potential risks to the foetus. Irregular foetal heartbeat, hypotonia, poor sucking and hypothermia in neonates have been reported following administration of either high doses or prolonged low dose administration of benzodiazepines in the third trimester or during labour. Bromazepam is excreted in breast milk. It is recommended that breast-feeding be discontinued during bromazepam therapy.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINES

The ability to carry out skilled tasks, such as driving or operating machinery, may be impaired to a varying degree depending upon dose, route of administration and individual response. Patients should be cautioned not to perform such tasks until they are sure they are not affected. Any such impairment will be potentiated by alcohol and alcohol should be avoided during therapy.

UNDESIRABLE EFFECTS

Cardiovascular system: Isolated reports of blood dyscrasias. Rare reports of hypotension and bradycardia.

Central Nervous System: The most common effects are ataxia, drowsiness, sedation, slurred speech and unsteadiness. These are dose related and may persist for over 24 hours even following single dose administration. Elderly patients may experience confusion, particularly if they have organic brain changes. More rarely headache and hallucinations, on occasion sexual in nature, may occur.

Dermatological System: Rare reports of skin rashes.

Gastrointestinal System: Rare reports of gastrointestinal upsets, dry mouth and increased appetite.

Hepatic System: Isolated reports of jaundice.

Immunological System: Hypersensitivity reactions, including anaphylaxis, to diazepam have rarely occurred.

Sensory Systems: Rare reports of vertigo and visual disturbances including blurred or double vision.

Urino-genital System: There are rare reports of urinary retention and changes in libido.

Patients who are elderly, or who have impaired hepatic and/or renal function are more likely to suffer from side effects. It is recommended that their therapy be regularly reviewed and be discontinued as soon as is practicable.

OVERDOSE

Symptoms: Intensification of therapeutic effects such as sedation, muscle weakness and profound sleep. In some cases it may be characterised by paradoxical reactions of excitation. High overdosage may result in apnoea, cardiac and respiratory depression, areflexia and coma. The possibility of there being more than one agent involved in cases of overdose should be considered.

Treatment: If ingestion is recent gastric lavage may be used to empty the stomach. Vital signs should be closely monitored. Treatment should be symptomatic and supportive as appropriate. In severe cases cardiovascular support and ventilation may be required. Excitation should not be controlled with barbiturates. The value of dialysis is unknown.

Flumazenil is a specific antidote for benzodiazepine overdose in emergency situations. It is administered intravenously and should only be used under experienced medical supervision in a hospital setting.

It should be noted that bromazepam overdose in conjunction with alcohol will be more severe and in the absence of adequate supportive measures may prove fatal.

SHELF LIFE

5 years

SPECIAL PRECAUTIONS FOR STORAGE

Store below 30°C in the original package, in order to protect from light.

NATURE AND CONTENTS OF CONTAINER

Tablets are packed in PVC/PE/PVDC-ALU (Perlalux) blisters. Packs of 1000 tablets are available.

PRODUCT REGISTRATION HOLDER

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MANUFACTURER

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